Dynamic Guideline: WHO-Based Malnutrition - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

WHO definition and clinical scope of malnutrition

WHO defines malnutrition as deficiencies, excesses or imbalances in nutrient intake, essential nutrients or nutrient utilization. WHO groups malnutrition into: **undernutrition**—including wasting, stunting and underweight; **micronutrient-related malnutrition**—including vitamin/mineral deficiencies or excesses; and **overweight, obesity and diet-related noncommunicable diseases** WHO Malnutrition fact sheet, WHO Malnutrition topic page.

For acute undernutrition, WHO defines **wasting** as low weight-for-height and notes that it often reflects recent severe weight loss and is associated with increased mortality risk if not treated properly. **Stunting** is low height-for-age and reflects chronic or recurrent undernutrition; **underweight** is low weight-for-age and may include children who are wasted, stunted or both WHO Malnutrition topic page. WHO Child Growth Standards define stunting, wasting and underweight as values below **−2 standard deviations** for the relevant anthropometric index WHO NLIS.

Exactly five WHO guideline or recommendation publications synthesized

#	WHO guideline / recommendation publication	Publication / release date identified	Target population	Scope and objectives	Relevance to malnutrition diagnosis, treatment or management
1	**WHO guideline on the prevention and management of wasting and nutritional oedema (acute malnutrition) in infants and children under 5 years**	2023; released November 2023 in related sources	Infants and children under 5 years, including infants under 6 months at risk of poor growth and children 6–59 months with moderate or severe wasting and/or nutritional oedema	Provides global, normative, evidence-informed recommendations and good practice statements for prevention and management of wasting and nutritional oedema WHO publication, NCBI Bookshelf Introduction	**Primary current WHO guideline** for acute malnutrition; supersedes the 2013 severe acute malnutrition guideline and expands to prevention, moderate wasting, severe wasting, nutritional oedema and care pathways BMJ Global Health
2	**WHO Guideline: updates on the management of severe acute malnutrition in infants and children**	2013; superseded by 2023 guideline	Infants and children, especially children 6–59 months with severe wasting and/or nutritional oedema	Identification and treatment of severe acute malnutrition; retained standing recommendations appear in the 2023 materials NCBI Bookshelf Introduction	Important historical foundation for antibiotics, vitamin A, diarrhoea/RUTF and post-discharge monitoring recommendations
3	**WHO/UNICEF recommendations for data collection, analysis and reporting on anthropometric indicators in children under 5 years old**	2019	Children under 5 years	Standardizes anthropometric data collection, analysis and reporting WHO NLIS	Supports diagnosis, surveillance and programme monitoring for wasting, stunting, underweight and overweight
4	**WHO guideline on fortification of edible oils and fats with vitamins A**	24 October 2025; launch surfaced 17	Population-level public health target; detailed	Fortification of edible oils and fats with vitamins A and D for public health	Addresses micronutrient-related malnutrition

#	WHO guideline / recommendation publication	Publication / release date identified	Target population	Scope and objectives	Relevance to malnutrition diagnosis, treatment or management
	and D for public health	November 2025	population not available in supplied summaries	WHO Malnutrition topic page	through food fortification
5	**Policies and interventions to create healthy school food environments: WHO guideline**	27 January 2026	School-age children and adolescents	Policies and interventions to shape healthy school food environments; schools are identified as critical settings for lifelong dietary habits WHO nutrition page	Relevant to prevention of unhealthy diets, micronutrient-related malnutrition and obesity; indirect relevance to undernutrition management

Most recent WHO youth-engagement guideline-development publication identified

The WHO publication titled **"Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)"** was identified as a WHO publication dated **11 December 2025** WHO Western Pacific publication page. It is **not a malnutrition guideline**; its relevance to this report is methodological, because it describes how youth input can be integrated into WHO guideline development through consultations and youth-led convening WHO webinar page.

1.2 Key Recommendations

A. WHO 2023 acute malnutrition guideline: principal recommendations and good practice statements available from supplied sources

The 2023 WHO guideline contains **21 recommendations**—14 new and 7 updated—and **12 good practice statements** NCBI Bookshelf Executive Summary. The table below includes recommendation text and grading that were verifiable from the supplied summaries.

Clinical area	Recommendation / good practice statement	GRADE Evidence Quality	Strength
Transfer from inpatient to outpatient care	Infants and children 6–59 months with severe wasting and/or nutritional oedema admitted to inpatient care can be transferred to outpatient care when danger signs have resolved for at least 24–48 hours, medical	Moderate	Strong

Clinical area	Recommendation / good practice statement	GRADE Evidence Quality	Strength
	problems no longer require inpatient care, appetite is good, oedema is resolving, and caregivers are linked to outpatient nutrition services. Transfer should **not** be based on anthropometric criteria alone such as specific MUAC or weight-for-height/length cut-offs NCBI Bookshelf table.		
Moderate wasting with risk factors	For infants and children 6–59 months with moderate wasting and specified risk factors—e.g., MUAC 115–119 mm, weight-for-age z-score < −3, age <24 months, relapse, failure to recover after other interventions, or history of severe wasting—prioritizing **specially formulated food interventions with counselling** rather than counselling alone should be considered MAGICapp WHO guideline PDF.	Moderate	Strong
Multiple micronutrient powders for prevention	In contexts where wasting and nutritional oedema occur, **multiple micronutrient powders should not be given to infants and children 6–23 months specifically to prevent wasting and nutritional oedema** 1,000 Days summary of WHO recommendation.	Moderate	Strong
Psychosocial stimulation	For children at risk of poor growth and development or with wasting and/or nutritional oedema, psychosocial stimulation should continue as part of care MAGICapp WHO guideline PDF.	Low	Conditional
Post-discharge monitoring	Children with severe wasting and/or nutritional oedema discharged from treatment programmes should be periodically monitored to avoid relapse NCBI Bookshelf table.	Low	Strong
Antibiotics in uncomplicated severe wasting/nutritional oedema managed outpatient	Children with uncomplicated severe wasting and/or nutritional oedema who do not require admission and are managed as outpatients should receive a course of oral antibiotic such as amoxicillin NCBI Bookshelf table.	Low	Conditional

Clinical area	Recommendation / good practice statement	GRADE Evidence Quality	Strength
Avoid routine antibiotics in undernourished children without severe wasting/nutritional oedema	Children who are undernourished but do not have severe wasting and/or nutritional oedema should **not** routinely receive antibiotics unless they show signs of clinical infection NCBI Bookshelf table.	Low	Strong
Vitamin A during treatment	Children with severe wasting and/or nutritional oedema should receive the daily recommended nutrient intake of vitamin A throughout treatment, either in therapeutic foods or in a multi-micronutrient formulation NCBI Bookshelf table.	Low	Strong
RUTF in children with diarrhoea	Children with severe wasting and/or nutritional oedema who present with acute or persistent diarrhoea can be given ready-to-use therapeutic food in the same way as children without diarrhoea, in inpatient or outpatient care NCBI Bookshelf table.	Very Low	Strong
Moderate wasting: nutrient-dense diet	Infants and children 6–59 months with moderate wasting should have access to a nutrient-dense diet to meet extra needs for recovery of weight and height and for survival, health and development NCBI Bookshelf Executive Summary.	Good practice statement; direct GRADE certainty not assigned	Good practice statement
Moderate wasting: comprehensive assessment	All infants and children 6–59 months with moderate wasting should be comprehensively assessed and treated where possible for medical and psychosocial problems causing or worsening wasting NCBI Bookshelf Executive Summary.	Good practice statement; direct GRADE certainty not assigned	Good practice statement
Prevention in affected contexts	Preventive interventions should ideally be implemented through a multisectoral and multisystem approach including food, health, WASH and social protection systems; they should include access to healthy diets, nutrition and medical services, counselling, maternal/family support and psychosocial care NCBI Bookshelf Executive Summary.	Good practice statement; direct GRADE certainty not assigned	Good practice statement

B. Diagnostic and classification criteria relevant to WHO-based management

Diagnostic / classification item	WHO-based definition or threshold	GRADE Evidence Quality	Strength
Wasting	Low weight-for-height; reflects acute undernutrition and is associated with increased mortality risk if not treated WHO Malnutrition topic page.	Technical definition; not GRADE-rated	Standard WHO definition
Stunting	Low height-for-age; reflects chronic or recurrent undernutrition and is associated with impaired physical and cognitive potential WHO Malnutrition fact sheet.	Technical definition; not GRADE-rated	Standard WHO definition
Underweight	Low weight-for-age; may include stunting, wasting or both WHO Malnutrition topic page.	Technical definition; not GRADE-rated	Standard WHO definition
WHO Child Growth Standards cut-off	Stunting, wasting and underweight are below −2 SD for height-for-age, weight-for-height and weight-for-age respectively WHO NLIS.	Technical definition; not GRADE-rated	Standard WHO definition
Severe wasting	WHO-hosted national guideline using WHO definitions reports severe wasting as MUAC <11.5 cm or weight-for-length/height <−3 z-scores WHO-hosted Lebanon national guideline PDF.	Technical definition; not GRADE-rated	Standard operational criterion
Moderate wasting	WHO-hosted national guideline reports moderate wasting as MUAC <12.5 cm or weight-for-length/height <−2 z-scores WHO-hosted Lebanon national guideline PDF.	Technical definition; not GRADE-rated	Standard operational criterion

1.3 Guideline Development Methodology

Evidence review process

The 2023 WHO guideline was prepared according to WHO guideline-development standards. It used systematic reviews and the **GRADE approach** to assess certainty of evidence and translate evidence into recommendations NCBI Bookshelf Methods. The guideline addressed intervention, diagnostic test accuracy and prognostic questions, and used GRADE Evidence-to-Decision frameworks MAGICapp WHO guideline PDF.

Consensus method

The 2023 WHO guideline used a Guideline Development Group with 27 members from all WHO regions, including research, programmatic and clinical expertise across the guideline's focus areas BMJ Global Health. The guideline development process was collaborative and transparent and applied GRADE methods to evidence synthesis and recommendation development BMJ Global Health.

Conflict of interest management

Potential conflicts of interest were managed case by case during development of the 2023 WHO guideline BMJ Global Health.

Good practice statements

WHO developed good practice statements using GRADE-guided criteria, including whether the statement was necessary for practice, had large net positive consequences, was clear and actionable, and whether direct evidence collection would be a poor use of resources NCBI Bookshelf Methods.

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Baseline WHO guideline for this update

The evidence update uses the **2023 WHO guideline on the prevention and management of wasting and nutritional oedema (acute malnutrition) in infants and children under 5 years** as the baseline guideline. This guideline superseded the 2013 WHO severe acute malnutrition guideline and expanded the scope to prevention, moderate wasting, severe wasting, nutritional oedema and integrated care pathways WHO publication, BMJ Global Health.

Search strategy

Evidence was summarized through **29 April 2026** using targeted searches for:

- WHO guidance and guideline annexes on wasting, nutritional oedema, acute malnutrition and GRADE.
- Peer-reviewed evidence after the 2023 WHO guideline, prioritizing systematic reviews, meta-analyses, randomized trials and major journals.
- Clinical trial registries for ongoing or recently completed trials of RUTF, RUSF, alternative therapeutic foods, psychosocial stimulation, relapse prevention, antibiotics, micronutrients, cash/voucher support and community management.

Databases and sources searched or represented

Source category	Sources represented in supplied evidence
WHO official guidance	WHO publication pages, WHO Malnutrition pages, WHO guideline PDFs, WHO MAGICapp annexes, NCBI Bookshelf WHO guideline chapters
Peer-reviewed journals	BMJ Global Health; Frontiers in Pediatrics
Data and technical standards	WHO Nutrition Landscape Information System
Clinical trial registries	ClinicalTrials.gov; ISRCTN

Source category	Sources represented in supplied evidence
Other authoritative summaries	WHO-hosted national guideline material; 1,000 Days summary of WHO prevention recommendation; Oxford Global Health publication listing

Inclusion criteria

Included evidence met one or more of the following:

1. Directly addressed **malnutrition**, **wasting**, **nutritional oedema**, **severe acute malnutrition**, **moderate wasting**, **micronutrient-related malnutrition**, or WHO anthropometric indicators.
2. Was published, released, registered or updated after the WHO 2023 guideline release and before or on **29 April 2026**.
3. Reported recommendations, evidence gaps, trial design, trial registration, systematic review context, or expert synthesis relevant to diagnosis, prevention, treatment or management.

Exclusion criteria

Excluded or not used for recommendation change:

- Records without verifiable malnutrition relevance.
- Post-2023 materials that did not report clinical outcomes, effect estimates or actionable changes.
- Trials without available results as of the evidence cutoff date.

2.2 Key New Studies

A. Post-guideline peer-reviewed and technical evidence

Study / source	Design	Key findings	GRADE Quality	Implication for WHO recommendations
BMJ Global Health 2025 article on development of the 2023 WHO guideline	Methods/reflections article	Described how the 2023 WHO guideline broadened scope to prevention and management of wasting and nutritional oedema across contexts and used systematic reviews and GRADE-informed methods BMJ Global Health 2025.	Low for direct clinical effectiveness; high relevance for methodology	Supports confidence in WHO process; does not provide new comparative clinical effect estimates.
BMJ Global Health 2025	Narrative guideline-focused article	Reported that the 2023 WHO guideline	Low for direct clinical	Reinforces implementation

Study / source	Design	Key findings	GRADE Quality	Implication for WHO recommendations
article on 2023 WHO guideline and integrated care		superseded the 2013 guideline and emphasized integrated care across inpatient, outpatient and community settings BMJ Global Health.	effectiveness	priority for integrated care pathways; no recommendation change.
BMJ Global Health article on evidence gaps in prevention	Evidence-gap/guideline synthesis	Highlighted that prevention of wasting and nutritional oedema was a significant addition to the 2023 guideline, but evidence gaps limited recommendations for several prevention interventions BMJ Global Health.	Low	Supports research need; no direct change to prevention recommendations.
Oxford Global Health listing of 2025 BMJ Global Health article on resource use and cost-effectiveness	Publication listing / economic evidence gap	Reported need for better evidence on resource use and cost-effectiveness of interventions for prevention and management of wasting and nutritional oedema Oxford Global Health.	Very Low for clinical effectiveness; Low for identifying economic evidence gaps	No recommendation change; strengthens need for cost-effectiveness studies.
Frontiers in Pediatrics 2026 expert-opinion review	Expert-opinion synthesis	Emphasized diagnostic anthropometry, etiologic assessment, phased care for severe acute malnutrition, early feeding, nasogastric	Very Low for comparative effectiveness	Consistent with WHO-style phased management; insufficient to modify WHO recommendations.

Study / source	Design	Key findings	GRADE Quality	Implication for WHO recommendations
		feeding when intake is inadequate, and recovery criteria including absence of oedema for at least 2 weeks Frontiers in Pediatrics.		
WHO Health Emergency Appeal 2026	WHO emergency technical report	Reported that disrupted nutrition services left **2.4 million people** facing severe acute malnutrition and described WHO-supported stabilization centres for children with severe acute malnutrition and medical complications in Sudan WHO Health Emergency Appeal 2026 PDF.	Low for intervention effectiveness; high relevance for burden and implementation urgency	Reinforces need for emergency implementation capacity; no change in clinical recommendation strength.

B. Clinical trial registry evidence through 29 April 2026

Trial / registry ID	Design / status from supplied evidence	Population and intervention	Key relevance	GRADE Quality
NCT06781918	Interventional Phase 3 trial; estimated start 3 February 2025; estimated study completion 14 May 2026	Children with severe acute malnutrition; new RUTF plus integrated psychosocial stimulation compared with look-alike supplement and standard local nutritional counselling; estimated enrollment 800 ClinicalTrials.gov NCT06781918.	Potentially important evidence on RUTF composition and psychosocial stimulation, but results were not available by 29 April 2026.	Very Low / no effect evidence yet

Trial / registry ID	Design / status from supplied evidence	Population and intervention	Key relevance	GRADE Quality
NCT06912620	ClinicalTrials.gov trial with protocol dated 16 April 2025 and statistical analysis plan dated 20 January 2026	Alternative RUTFs for treatment of child wasting; keywords include severe acute malnutrition, moderate acute malnutrition, plant-based, dairy-free and peanut-free RUTFs, and wasting relapse ClinicalTrials.gov NCT06912620, SAP PDF.	Important active research on alternative RUTF formulations and relapse prevention; no completed outcome results available.	Very Low / no effect evidence yet
ISRCTN53213318	Randomized trial with intervention details updated 26 August 2025; follow-up/sample collection continuing to end August 2026	Moderately undernourished young children recovering from illness; RUSF, locally available food, microbiome-directed supplementary food, counselling and monthly food vouchers ISRCTN53213318.	Relevant to recovery sustainability, supplementary foods and food vouchers; no final results in supplied evidence.	Very Low / no effect evidence yet
ISRCTN60973756	Registry record; dates not supplied in search summary	Severe acute malnutrition treatment delivered by community health workers in emergency settings in Mali; linked to RUTF and CMAM ISRCTN60973756.	Relevant to community delivery and access; no outcome results in supplied evidence.	Very Low / no effect evidence yet
NCT05994742	Adaptive multi-arm trial; dates not supplied in search summary	Children recovering from complicated severe acute malnutrition; standard RUTF, psychosocial support, medications including cotrimoxazole prophylaxis and ART for children with HIV ClinicalTrials.gov NCT05994742.	Relevant to complicated SAM recovery and adjunctive interventions; no outcome results in supplied evidence.	Very Low / no effect evidence yet

2.3 Evidence Gaps and Future Research Needs

The 2023 WHO guideline itself identified major evidence gaps; WHO's introduction states that more evidence is needed to determine how best to prevent and manage wasting and nutritional oedema and that the guideline will be responsive and data-driven as evidence becomes available NCBI Bookshelf Introduction. BMJ Global Health similarly reported that evidence gaps made it difficult for the Guideline Development Group to make recommendations for all prioritized questions and interventions BMJ Global Health.

Priority evidence gaps

Domain	Evidence gap	Current evidence certainty	Research priority
Prevention	Limited direct evidence for many prevention interventions, including fortified blended foods, lipid-based supplements, cash transfers and food vouchers in specific contexts NCBI Bookshelf prevention annex.	Low to Very Low	Large pragmatic trials and implementation studies stratified by food insecurity, disease burden, seasonality and baseline wasting prevalence
Moderate wasting	Uncertainty about which children benefit most from specially formulated foods versus counselling and locally available nutrient-dense diets	Moderate for selected risk-factor group; lower outside that group	Comparative effectiveness studies by MUAC, WAZ, age, relapse history and comorbidity
Severe wasting and nutritional oedema	Evidence gaps remain for optimal inpatient/outpatient criteria, treatment of complications, relapse prevention and follow-up models	Low to Moderate depending on question	Trials and cohorts evaluating simplified protocols, integrated care pathways and post-discharge monitoring
RUTF formulation and dosing	Ongoing trials are evaluating alternative RUTFs, but completed post-2023 results were not available in supplied evidence ClinicalTrials.gov NCT06912620.	Very Low post-guideline evidence	Non-inferiority and superiority trials assessing recovery, relapse, acceptability, cost and safety
Psychosocial stimulation	WHO recommendation is conditional with low-certainty evidence; new trials are ongoing ClinicalTrials.gov NCT06781918.	Low	Trials measuring neurodevelopment, growth, survival, caregiver outcomes and feasibility
Antibiotics	WHO standing recommendations rely on low-certainty evidence; no new antibiotic-focused post-2023 trial was identified in supplied evidence	Low	Context-specific trials considering antimicrobial resistance, HIV burden, inpatient/outpatient status and infection risk
Cost-effectiveness	Direct cost-effectiveness evidence is limited; economic evidence gaps	Very Low to Low	Embedded economic evaluations in clinical and implementation trials

Domain	Evidence gap	Current evidence certainty	Research priority
	were highlighted in 2025 Oxford Global Health.		
Humanitarian settings	Severe acute malnutrition burden remains high in emergencies; service disruption is a major risk WHO Health Emergency Appeal 2026 PDF.	Low	Operational research on stabilization centres, supply chains, community delivery and continuity of care

2.4 Updated Recommendations Based on New Evidence

No completed post-2023 randomized trial, systematic review or meta-analysis in the supplied evidence provided sufficient new outcome data to overturn or materially change the 2023 WHO recommendations by **29 April 2026**. Therefore, the WHO 2023 guideline remains the primary actionable standard.

Original WHO recommendation / guidance area	New evidence impact through 29 April 2026	Updated recommendation
Use WHO 2023 integrated prevention and management framework for wasting and nutritional oedema in children under 5	Post-guideline BMJ Global Health articles reinforce the guideline's expanded scope and integrated care emphasis, but do not provide new effect estimates BMJ Global Health.	**Maintain WHO 2023 recommendation framework.** Implement integrated inpatient, outpatient and community care pathways.
Transfer from inpatient to outpatient care should be based on clinical stabilization, appetite, resolving oedema and linkage to outpatient services, not anthropometry alone	No new post-2023 comparative evidence identified.	**Maintain strong recommendation, moderate-certainty evidence.**
Prioritize specially formulated food plus counselling for moderate wasting with specified risk factors	No completed new trial evidence identified; ongoing and registry studies may inform future updates.	**Maintain strong recommendation, moderate-certainty evidence** for the specified high-risk moderate wasting group.
Do not use multiple micronutrient powders specifically to prevent wasting/nutritional oedema in children 6–23 months in affected contexts	No new evidence identified to reverse this recommendation.	**Maintain strong recommendation, moderate-certainty evidence.**

Original WHO recommendation / guidance area	New evidence impact through 29 April 2026	Updated recommendation
Continue psychosocial stimulation for children at risk of poor growth/development or with wasting/nutritional oedema	Ongoing Phase 3 trial NCT06781918 may provide future evidence; no results available by cutoff ClinicalTrials.gov NCT06781918.	**Maintain conditional recommendation, low-certainty evidence.** Reassess when trial results become available.
Provide post-discharge monitoring to avoid relapse	Registry evidence shows active interest in relapse prevention, including alternative RUTFs and voucher-supported follow-up, but no completed results were available ClinicalTrials.gov NCT06912620, ISRCTN53213318.	**Maintain strong recommendation, low-certainty evidence.** Strengthen implementation and data collection on relapse.
Use RUTF for severe wasting/nutritional oedema, including in children with diarrhoea as recommended	Alternative RUTF trials are underway, but no completed post-2023 results were available ClinicalTrials.gov NCT06912620.	**Maintain WHO recommendation.** Do not replace standard RUTF protocols solely on the basis of registry evidence.
Oral antibiotics such as amoxicillin for uncomplicated severe wasting/nutritional oedema managed outpatient	No new antibiotic trial result identified; antimicrobial-resistance implications remain important.	**Maintain conditional recommendation, low-certainty evidence** while applying local infection and antimicrobial stewardship policies.
Avoid routine antibiotics in undernourished children without severe wasting/nutritional oedema unless clinical infection is present	No new evidence identified to change this.	**Maintain strong recommendation, low-certainty evidence.**

Practical WHO-Based Clinical Management Summary

Screening and diagnosis

1. **Measure and classify** using WHO Child Growth Standards:
    - Weight-for-height/length z-score.
    - Mid-upper arm circumference where applicable.
    - Presence and grade of bilateral pitting oedema.
    - Weight-for-age and height-for-age to identify underweight and stunting.
2. **Identify severe acute malnutrition / severe wasting and/or nutritional oedema**:
    - Severe wasting commonly operationalized as MUAC <11.5 cm or weight-for-height/length <−3 z-scores, or presence of nutritional oedema WHO-hosted Lebanon national guideline PDF.

3. **Assess for complications**:
    - Danger signs.
    - Appetite.
    - Infection.
    - Dehydration.
    - Diarrhoea.
    - HIV, tuberculosis or other chronic disease where epidemiologically relevant.
    - Psychosocial and feeding problems.

Treatment and management

1. **Severe wasting/nutritional oedema with complications**:
    - Manage initially in inpatient care until clinically stabilized.
    - Transfer to outpatient care only when WHO clinical criteria are met and outpatient nutrition services are linked NCBI Bookshelf table.
2. **Uncomplicated severe wasting/nutritional oedema**:
    - Outpatient management with therapeutic feeding where feasible.
    - Oral antibiotic such as amoxicillin is conditionally recommended for uncomplicated severe wasting/nutritional oedema managed outpatient NCBI Bookshelf table.
3. **Moderate wasting**:
    - Ensure access to nutrient-dense diets and comprehensive medical and psychosocial assessment NCBI Bookshelf Executive Summary.
    - For children 6–59 months with moderate wasting plus high-risk features, consider specially formulated foods plus counselling MAGICapp WHO guideline PDF.
4. **Relapse prevention**:
    - Monitor after discharge.
    - Provide feeding counselling, recognition of illness, care-seeking support, responsive care and WASH-related support where feasible MAGICapp WHO guideline PDF.
5. **Population-level prevention**:
    - Use multisectoral strategies including food systems, health services, WASH, social protection, breastfeeding support, healthy diets and psychosocial care NCBI Bookshelf Executive Summary.

Overall Conclusion

As of **29 April 2026**, the **2023 WHO guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years** remains the central WHO-based guideline for acute malnutrition diagnosis, treatment and management. The post-guideline evidence identified through the cutoff date mainly consists of guideline-methods articles, evidence-gap analyses, expert-opinion synthesis and ongoing clinical trials. No completed post-2023 trial or meta-analysis in the supplied evidence was sufficient to change WHO’s core recommendations. Therefore, current practice should continue to implement WHO

2023 recommendations while prioritizing research on prevention, RUTF formulations, psychosocial stimulation, relapse prevention, community delivery, antibiotics and cost-effectiveness.